Meropenem

FORM Meropenem 500mg powder for solution for injection vials

INDICATION Management of severe aerobic and anaerobic gram-positive and gram-negative infections as directed by neonatologist or microbiologist.

DOSE RANGE

AGE	DOSE*	FREQUENCY	ROUTE
<7 days	40 mg/kg	12 hourly	IV injection or infusion
7 to 28 days	40 mg/kg	8 hourly	IV injection or infusion
1 to 6 months	40 mg/kg	8 hourly	IV injection or infusion

**Dose adjustment required in renal impairment*

For positive cultures where the isolate is reported as 'I' an increased exposure dosing regimen is required. Exposure may be increased as a function of dose, frequency or administration time depending on the drug.
Please refer to the following guidance *"Isolates reported as 'I' (susceptible increased exposure)- dosing schedules for neonates and children (944)*

RECONSTITUTION

Reconstitute one 500mg vial with 9.5 ml water for injection to give 500 mg in 10 ml (50 mg in 1 ml) After adding the diluent, shake the vial well and allow to stand until the solution is clear. If this take more than 1 minute, shake again then allow to stand for a further minute. Repeat until powder has dissolved.

Dosage guide:

- 500 mg in 10 ml
- 50 mg in 1 ml
- 5 mg in 0.1 ml

DILUTION

IV bolus: give neat

IV infusion: dilute with sodium chloride 0.9% to a concentration between 1mg per ml and 20mg per ml.

Example; for a 50mg dose for IV infusion
- Draw up 1ml of reconstituted solution. Further dilute to 5ml with sodium chloride 0.9%
- Run at 10ml/hr over 30mins

METHOD OF ADMINISTRATION

IV bolus over 5 minutes
IV infusion over 30 minutes

Neonatal

COMPATIBILITY

Solution compatibility	Sodium chloride 0.9% Glucose 5%,
Solution incompatibility	All other fluids
IV Line compatibility	For IV bolus administration: TPN, furosemide, insulin, morphine, paracetamol, potassium chloride For IV infusion discuss compatibility with pharmacy
IV Line incompatibility	Sodium bicarbonate, phosphate preparations

THIS LIST IS NOT EXHAUSTIVE PLEASE CONTACT PHARMACY FOR FURTHER INFORMATION ON COMPATIBILITY WITH ANY MEDICINES NOT INCLUDED

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS

- See Summary of Product Characteristics and most recent edition of BNF for Children

FURTHER INFORMATION

- A positive direct or indirect Coombs test may develop in some patients during treatment with meropenem.
- In patients with hepatic disease, transaminases and bilirubin should be monitored. No dose adjustment is necessary.
- For information on extended infusions see: Isolates reported as 'I' (susceptible increased exposure) - dosing schedules for neonates and children (944) | NHSGGC

PH

7.3 - 8.3

LICENSED STATUS

Not licensed in children < 3 months old

APPLICABLE POLICIES

West of Scotland Neonatal Guidelines
Consult local policy if applicable

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Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department